



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : اميره عبدالنواب علي سيد
المحافظة : الجيزة
الحى / المدينة : البدرشين
رقم البطاقة : 28609182101265 رقم التليفون : 01225061968
العنوان : قرية العزيزيه
العمر (تاريخ الميلاد) : 18-09-1986 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد [14112] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: اميره عبدالنواب علي سيد National ID: 28609182101265	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Trastuzumab Subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



اميره
عبد القواب على سيد احمد
قرية العزيزيه
مركز البدرشين - الجيزه

١٩٨٩/١١/٢٨ ٢٨٦.٩١٨٢١.١٢٦٥
J18497531

٢٠٢٢/٠٥ ٢٨٦.٩١٨٢١.١٢٦٥ ربة منزل



أنثى
مسلمة
متزوجة
سلامه مطاوع الويكر-محمد
البطاقة سارية حتى ٢٠٢٩/٠٥/٠٦



Patient Name: Amira Abd Al Tawwab Ali	Date : 03/11/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 1034/2025	Pat. ID: 99267

BREAST HIGH RESOLUTION ULTRASONOGRAPHY REVEALED:

Indication: Follow up after neoadjuvant chemotherapy of pathologically proved left breast carcinoma.

- Homogenous fatty background parenchyma.
- The previously described malignant mass seen at 1 o'clock position of the left breast ,zone 1-2 ,now measures (13 x 12 mm) and the other similar one at 1 oclock position zone 1-2 now measures (8.5 x 5.5 mm) .
- No other suspicious masses in other quadrants.
- No detected suspicious masses in the right breast .
- Normal skin, subcutaneous fat and nipple areola complex on both sides.
- No significant axillary lymph nodes.

Conclusion:

- Left breast (BIRARDS 6 category).
- Normal study of the right breast (BIRADS 1 category).

Dr farah elshobaky

Name: اميره عبدالنواب علي			
MR. : 253			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
		تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 38-year-old individual with a left-sided breast carcinoma, clinical stage T4 N2, grade not specified. Hormone receptor testing showed estrogen receptor 8/8 positive, progesterone receptor 5/8 positive, and HER2 3+ positive. Neoadjuvant chemotherapy was administered consisting of four cycles of docetaxel (Taxotere) and carboplatin, followed by two cycles of PHESGO (pertuzumab, trastuzumab, and chemotherapy) as neoadjuvant therapy. Anti-HER2 therapy had not yet been given at the time of surgical consultation. The patient underwent a left modified radical mastectomy (MRM) on 2 April 2026. The multidisciplinary team (MDT) discussion on 11 May 2026 noted that the patient had completed two cycles of PHESGO neoadjuvant therapy and remained clinical stage T4 N2. The MDT recommended adjuvant HER2-directed therapy with trastuzumab (Herceptin) and ovarian suppression with zoladex, pending confirmation of anti-HER2 receipt. An echocardiogram performed on 6 June 2026 showed a left ventricular ejection fraction (EF) of 65 %. On 29 June 2026, the internal medicine note reiterated the same disease characteristics and treatment history, confirming two cycles of PHESGO neoadjuvant therapy and the plan to seek trial approval for adjuvant PHESGO to complete one year of therapy, together with zoladex and postoperative radiotherapy. The note also recorded an EF of 65 % on 6 June 2026 and outlined the upcoming schedule: cycle 2 of trastuzumab (Herceptin) 600 mg subcutaneously, cycle 3 with trastuzumab plus letrozole (Femara), and subsequent cycles as planned. The note dated 20 July 2026 detailed the administration of cycle 2 trastuzumab subcutaneously together with letrozole, and indicated that zoladex was to be continued. Pharmacy orders were placed for cycle 3 on 3 August and for cycle 4 on 4 August, with trastuzumab subcutaneously, letrozole, and clinic follow-up including an echocardiogram for cycle 4. On 10 August 2026, the pharmacy record confirmed the dispensing of trastuzumab 600 mg subcutaneously and letrozole for cycle 3, with reference to the echocardiogram performed on 6 June 2026 showing an EF of 65 %. The pharmacy note on 31 August 2026 documented the dispensing of letrozole and referenced an echocardiogram performed on 31 August 2026 with an EF of 25 %, and indicated that trastuzumab therapy should be stopped. The most recent oncology note on 5 September 2026 reported a further decline in left ventricular ejection fraction to 3.5 % (1/8). The treating physician recommended omitting the implanted port given the left-sided location and severely reduced EF, and advised repeating an echocardiogram after three weeks. The current plan is to withhold further trastuzumab and to monitor cardiac function closely.			
تحريراً في ,			
		التاريخ :	21-09-2026
		الوقت :	
		الطبيب المعالج :	dina makram
يعتمد ,			

Patient Name : الأستاذ / اميره عبد التواب على سيد
Patient No. : 28609182101265
Path No. : 865/26
Sex : Female
Age : 39 Y
Referring Doc :

Request No. : 13005685
Request Date : 02/04/2026
Verification Date : 07/04/2026
Financial Category : جوسف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Axillary lymph nodes: Fourteen lymph nodes free of viable metastatic deposits, LNs (0/14) (Three lymph nodes showing therapy effect).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature:

Dr. Walaa Ibrahim Ghoneim.

Dr. Duaa Mohamed.

Verified by :

Dr.Duaa Mohamed

Printing Date : 14/04/2026 12:59:51

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٢٥٢

Patient Name : الأستاذه / اميره عبد التواب على سيد
Patient No. : 28609182101265
Path No. : 865/26
Sex : Female
Age : 39 Y
Referring Doc :

Request No. : 13005685
Request Date : 02/04/2026
Verification Date : 07/04/2026
Financial Category : جوستاف روسي - افقة الدولة
Department : Out Patient
Registered by : Mrs.Mâyada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 22x20x5 cm, with covering skin ellipse measured 20x17 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 8x7x3 cm, located 2 cm from the least deep margin.
- * Axillary fat measured 8x6x2 cm, entangling 14 lymph nodes, the largest measured 1.5x1x0.5 cm.
- * 12 Blocks:
1:6- T.B.S., 7- N&A., 8- Deep margin., 9:12- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy (Hyalinosis & microcalcification).
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Fourteen lymph nodes free of viable metastatic deposits, LNs (0/14) (Three lymph nodes showing therapy effect).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of the previously diagnosed invasive tumor.

Verified by :
Dr.Duaa Mohamed

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04/20/2026

MDT FORM

Name:	Amira Abdeltawab Aly .	MRN:	253
Age:	40 Y	ID:	26609182101265
Menopausal Sta	PRE	Phone:	012806630167
NRS SCORE:		Marital Status:	
BMI:		Family History:	-VE
CO Morbidity:	M.FREE	Reason Of MDT:	POT OP

Clinical Data:

BX:	IDC G2
ER: 8/8/	PR: 5/8 HER2: 3+ KI67: 20%

R

L

Sonomammography:

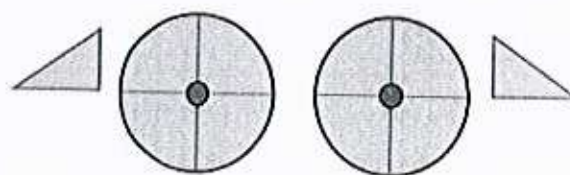
SP LT MRM POST NACT
DCIS ABSENT
NIPPLE&AREOLA FREE
DEEP MARGIN FREE
AXILLARY LN (0\14)
PATHOLOGICA; COMPLETE RESPONSE

Mets Workup:

INITIALLY T4B N2

History:**Recommendations:****MDT Outcome & Plan**

ADJUVANT HERCEPTAN (NO ANTI HER2 received as NACT)
ZOLADEX +AI
PORT



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نرمن مصطفى					



إدارة رئيس الجمهورية
لصحة المرأة المصرية

المنشأة المصرية
للصحة
مصر

MDT FORM

26/5/2025 G C

Name	أ.م.د. عبد الوهاب	MRN	1034/2025
Age	38	ID	517.9121110
Menopausal status	pre-menopausal	Phone	0122.5061968
NRS SCORE		Marital status	Married
BMI		Family history	ve
CO morbidity	NAD	REASON OF MDT	New Case

Clinical Data

BX: IDC of Follicular DCIS

ER: 8/8 PR: 5/8 HER 2: 1.3 KI67: 25%

Sonomammography: area of Distortion
e' no Distinctive mass at 2 o'clock
e' Prominent Lt Axillary Lvs

Mets Workup:
Met workup:- free

R

L



TNM

Recommendations:

MDT OUTCOME & PLAN

BS
New SM
ACX4 + 4 TCHP

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name : الأستاذة / اميرة عبد الثواب على سرمد	Request No. : 13003820
Patient No. : 28609182101265	Request Date : 19/02/2025
Path No. : 726/25	Verification Date : 05/03/2025
Sex : Female	Financial Category : أورام دار السلام - مبانى
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast area of distortion.

Nature of Specimen: Tru-cut biopsy, collected at 18/2/2025.

Gross Examination:

- 3 grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for Immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal DCIS (comedo pattern).
- * Perineural invasion: No.
- * Angiolinvasion: No.
- * There is dystrophic calcification.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
- PR: Strong nuclear reaction in 9% of tumor cells (score 5 /8).
- Her2: Strong complete membrane reaction in > 10% of tumor cells (score 3+).
- Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:

Left breast area of distortion, Tru-cut biopsy:
Invasive duct carcinoma, Grade II and focal DCIS.

IPT:

- ER: Positive (score 8 /8).

Verified by :

DR.Menna Nabil

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Patient Name : الأمينة / اميره عبد التواب على سيد	Request No. : 13003820
Patient No. : 28609182101265	Request Date : 19/02/2025
Path No. : 726/25	Verification Date : 05/03/2025
Sex : Female	Financial Category : مبلره - اورام دار السلام
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- PR: Positive (score 5 /8).
- Her2: Positive (score 3+).
- Ki67: Positive In 25% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.

Dr. Menna Nabil.

Verified by :

DR.Menna Nabil

Printing Date : 07/04/2025 11:09:13

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Patient Name: Amira Abd Al Tawwab Ali	Date : 29/06/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 38 Year(s)
Medical ID: 1034/2025	Pat. ID: 99267

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- **Breast density:** Scattered areas of glandular tissue in both breasts... *Matching ACR b.*
- **A large area of distortion with extensive pleomorphic calcifications is seen in the left upper outer quadrant over an area of 6 cm ,another small grouped pleomorphic calcifications is seen in the left retroareolar region (distance from the area of distortion by 3 cm)..**
- No detected definite suspicious mass density, or pathological micro-calcification in the right breast.
- Normal skin and nipple the right side .

By complementary breast ultrasound:

- Homogenous (predominantly glandular) back ground parenchymal echo-texture.
- Thickened left breast skin (6 mm)
- An irregular shaped mass is seen at 1 o'clock position of the left breast , zone 1-2 , measuring 15 x 13 mm ,another similar mass is seen at 1 o'clock ,zone 1-2 measuring (11 x 10 mm).
- An area of parenchymal heterogeneity & distortion is seen in-between 12-2 o'clock position of the left breast zones 2..
- Multiple(more than 3 in number) suspicious looking left axillary lymph nodes ,level 1 & 2,the largest measures (17 x 17 mm) with thick cortex of 7mm .

CONCLUSION:

- Left breast : locally advanced UOQ multifocal suspicious masses and area of distortion reaching the nipple areolar complex (**BI-RADS 6 category**) .
- Right breast : normal study : (**BI-RADS 1 category**)

MUCH OBLIGED

DR.MONA ZAKY, MD.

Professor of diagnostic radiology -Breast Imaging-Mansoura University

Signature Dr. Mona Zaki

29/06/2025 2:50 PM



Patient Name: Amlra Abd Al Tawwab Ali	Date : 03/05/2026
Exam : C.T. CHEST	Age : 38 Year(s)
Medical ID: 1034/2026	Pat. ID: 89267

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left breast cancer.

I Chest:

- Left breast retroareolar asymmetry with diffuse skin thickening.
- Enlarged left axillary level I and II lymph nodes the largest measures 8 x13 mm.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.

ECHO REPORT

NAME : أميرة عبد التواب علي DATE: 3 / 11 / 2025
ID NUMBER : ١٠٣٤ / ٩٥ SEX : ♀ AGE: 38

EF	68%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVEDD	2.3	2-4cm	LA	3.7	2-4cm
FS	28%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RWMA
LA and Ao Root Dimensions Normal
Mitral Valve N
Aortic Valve N
Tricuspid Valve N
Pulmonary Valve
Right Side and Systolic Function TAPSE: 2.5
Intracardiac Masses and Effusion

DR. 



أمية في الطب

Dear Prof

Mrs Amira is a case of ER+/HER⁺
left DC CTa b No No,
received NACT + phos go →
MRH (ypt No) - she developed
Heart Failure (with recent
pleurisy CT showing mild pleural
effusion). In view of MRH
done & in view of cardiac
risk the left sided Rth & in
view of the Bst study results,
we prefer to omit PORT.



أمية في الطب

Dear Prof

Mrs Amira is a case of ER+/HER⁺
left DC CTa b No No,
received NACT + phos go →
MRH (ypt No) - she developed
Heart Failure (with recent
pleurisy CT showing mild pleural
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we prefer to omit PORT.